KinPath: Immediate Next Steps Plan

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Core objective

Do not try to build the entire KinPath vision first. Your immediate goal is to prove that Atlanta hospice organizations will adopt and ultimately pay for a narrow product: a bereavement-operations workflow for staff plus a calm, structured handoff for families after a death.

First product thesis:

> KinPath helps hospice teams manage bereavement follow-up and gives families a guided plan for what comes next.

This is narrower and more fundable than "a funeral app" or a broad "next-of-kin operating system." It gives you one buyer, one high-value workflow, one measurable problem, and one clear route to early revenue.

What not to do first

- Do not build separate native iOS and Android apps before validating that a secure responsive web product is not enough.
- Do not launch a nationwide death-doula marketplace in the first year.
- Do not build insurer claims integrations, HRIS integrations, hospital modules, or a full employer PEPM product before validating the hospice product.
- Do not build a full pre-need estate-planning platform yet.
- Do not lease a dedicated office, hire a large team, run paid advertising, or retain an expensive agency before you have customer commitments.
- Do not make unverified claims about time savings, productivity, clinical outcomes, benefits eligibility, HIPAA status, or regulatory compliance.

90-day outcome

By day 90, you should be able to say:

> "We interviewed 25–30 people, mapped five real bereavement workflows, signed three hospice design partners, tested a clickable prototype, established pilot metrics, and know exactly what a narrow secure MVP must prove."

That is the point at which KinPath becomes fundable rather than merely promising.

90-day roadmap

Timeframe	Primary outcome	Required deliverables
Days 1–30	Prove the problem and narrow the first workflow	15+ interviews, workflow map, product brief, clickable prototype, initial legal/compliance guidance
Days 31–60	Turn interest into committed design partnerships	25–30 interviews total, 3–5 LOIs, pilot scorecard, pitch deck, initial budget, accelerator/grant applications
Days 61–90	Prepare funding and build the smallest credible pilot	Pilot agreements, implementation plan, fundraising package, scoped MVP proposal, technical lead or vendor selection

Days 1–7: Set the foundation

Day 1: Write the one-page product brief

Create a single-page document that every prospective partner, advisor, developer, and investor can understand.

Include:

- **One-sentence description:** "KinPath helps hospice teams manage bereavement follow-up and gives families a guided plan for what comes next."
- **First buyer:** Georgia Medicare-certified hospice operators that have a meaningful annual death volume, a bereavement coordinator or similar role, and a manual or fragmented follow-up workflow.
- **First user:** Bereavement coordinator, hospice social worker, chaplain, administrator, or clinical leader.
- **First family promise:** "In the first five minutes, you will know what needs attention now, what can wait, and who can help."
- **First workflow:** Death notification, family invitation, outreach scheduling, contact logging, urgent task plan, document requests, and survey-ready reporting.
- **Problem statement:** Manual calls, spreadsheets, disconnected calendars, inconsistent family handoffs, and time-consuming documentation create staff burden and an uneven family experience.
- **Explicit boundaries:** KinPath is not legal advice, medical advice, licensed funeral-arrangement service, emergency care, or a substitute for an attorney, clinician, funeral director, or hospice professional.

Day 2: Build a hospice prospect list

Create a spreadsheet with 40 Atlanta-area hospice prospects.

For each organization, capture:

- Organization name
- Website

- Geographic coverage and locations
- Estimated scale, if publicly available
- CEO, administrator, COO, clinical leader, bereavement coordinator, social-work leader, or chaplaincy leader
- Email and LinkedIn profile
- Whether you have a warm introduction
- Date of first outreach
- Discovery-call status
- Key notes and follow-up date

Prioritize warm introductions through your consulting network, professional contacts, faith communities, healthcare contacts, senior-care contacts, and local founder ecosystem.

Day 3: Prepare customer-discovery materials

Create:

- A 30-minute discovery-interview script
- A brief email explaining that you are learning about workflows, not selling a finished product
- A confidentiality statement explaining that you do not want patient-identifiable information during early research
- A consistent interview-notes template
- A simple CRM or tracking sheet

Use this note template after every call:

- Interviewee, title, organization, and date
- Current workflow from death notification through 12-month follow-up
- Tools used today
- Biggest workload drivers
- Reporting/compliance pain points
- Family questions that repeat most often
- Existing workarounds
- Budget owner and purchasing path
- Exact quotes
- Follow-up action

Days 4–7: Begin outreach

- Send 10–15 personalized messages to potential hospice contacts.
- Ask for a 25-minute learning conversation, not a sales meeting.
- Schedule at least five interviews for the following two weeks.
- Book one limited-scope consultation with a startup, healthcare, privacy, or technology attorney, or identify a legal clinic that can provide initial guidance.
- Create a simple landing page describing the problem and inviting hospice teams to speak with you or become a founding design partner.

Use this outreach language:

> I am researching how hospice teams manage bereavement follow-up, family questions, outreach documentation, and the transition after a patient dies. I am building KinPath, a possible workflow and family-guidance product, but I am not selling a finished product. Would you be open to a 25-minute conversation about your current process and where it creates the most staff burden?

Days 1–30: Validate the problem

Interview targets

Group	Target	What to learn
Hospice bereavement coordinators, social workers, chaplains	10	Current workflow, caseload, manual work, family questions, documentation gaps
Hospice executives and administrators	5	Budget owner, compliance concern, procurement, implementation requirements
Recently bereaved family members	5	Confusing tasks, technology preferences, family coordination needs, trust barriers
Death doulas or care navigators	5	Common family task patterns, care boundaries, handoff needs, service-quality expectations
Optional HR/benefits leaders	5	Later-market insight only; do not allow this to distract from the hospice wedge

Core discovery questions

Ask hospice teams:

1. Walk me through what happens from the moment a patient dies through the next 12 months of family follow-up.
2. Who owns each part of the workflow, and where do handoffs fail?
3. What systems do you use today: EHR, spreadsheet, shared inbox, calendar, call log, paper, or something else?
4. Which activities consume the most staff time?

5. What is hardest to document or report?
6. Which family questions recur most often in the first days and weeks?
7. What work is most likely to be delayed, missed, or inconsistently completed?
8. What would make a new tool difficult for staff to adopt?
9. Who would approve a purchase and which budget would fund it?
10. What result would make a new system worth paying for?
11. If you could eliminate one part of the current process tomorrow, what would it be?

Discovery rules

- Ask about past behavior and current workflow, not hypothetical interest in a feature list.
- Listen for workarounds. Spreadsheets, sticky notes, manual call lists, shared inboxes, and personal calendars are evidence of a real problem.
- Record exact phrases. Your future website, deck, and product copy should use the customer's language.
- Do not request or store protected health information during early discovery.
- Write a one-page interview summary within 24 hours of each conversation.
- At the end of every call, ask who else you should interview and whether the person would review a prototype later.

Map five real workflows

By day 30, create a visual current-state workflow map based on interviews. It should show:

- Trigger: death notification
- Who is notified internally
- How the family is identified and contacted
- How family consent and preferences are captured
- Outreach cadence and channels
- Documentation process
- Escalations to clinical/support staff
- Family information and administrative resources
- Reporting, audit, and survey preparation
- Points where staff re-enter data or lose context
- Points where families are most confused or unsupported

Your goal is to identify the one workflow that is painful, common, measurable, and realistic for KinPath to improve.

Product prototype: Build before code

Create a clickable prototype

Use Figma, Canva, a no-code prototype tool, or another inexpensive option. Do not commission production software until design partners validate this flow.

The prototype should demonstrate one complete case:

1. A staff member receives a death notification.
2. The case enters the hospice dashboard.
3. The staff member sees the next required outreach, outstanding tasks, and contact history.
4. The family receives a secure invitation.
5. The family sees a calm “first 72 hours” guide.
6. The family can delegate a task, receive a document request, and understand what can wait.
7. The staff member records outreach and sees case status.
8. The hospice can export a simple, survey-ready activity report.

Required prototype screens

- Staff case list with due, overdue, completed, and high-priority indicators
- Case timeline with outreach attempts, next contact date, tasks, and audit history
- Configurable outreach cadence
- Family invitation by email, text, or secure link
- Family home screen: “Do now,” “Can wait,” and “Who can help”
- Simple delegated tasks
- Document request and receipt tracking
- Contact log
- Report/export concept
- Clear privacy and service-boundary notices

Do not add these yet

- Vendor booking or payment
- Featured listings, commissions, referral fees, or marketplace economics
- Automated benefits eligibility decisions
- Claim filing on behalf of families
- Complex family voting mechanisms
- EHR, HRIS, EAP, insurer, or claims-system integrations
- Clinical therapy delivery

- Will drafting or probate advice

Days 31–60: Secure design partners

Goal: 3–5 written design-partner commitments

A design partner is not simply someone who says the idea is good. It agrees to help you shape and evaluate a specific pilot.

Ask each design partner for:

- A named executive sponsor
- A named frontline champion such as a bereavement coordinator, social worker, chaplain, or administrator
- Two to four workflow and prototype-feedback sessions
- Permission to gather de-identified baseline measures
- A written letter of intent or pilot agreement
- A defined pilot evaluation period
- A conversion conversation after the pilot
- Permission to pursue an anonymized or approved case study if success targets are met

Offer in return:

- Founding-partner pricing or a tightly limited pilot
- High-touch founder-led onboarding
- Configuration around the hospice's actual outreach process
- Rapid feedback incorporation where feasible
- A documented implementation plan
- Clear privacy and data-boundary practices

Design-partner ask

Use this language:

> I am seeking three Atlanta hospice design partners to shape and evaluate KinPath, a bereavement-operations and family-guidance platform. The pilot would focus narrowly on follow-up scheduling, contact documentation, staff visibility, and a family handoff. In exchange for your workflow input and measured feedback, KinPath would provide founder-level implementation support and founding-partner terms. If the agreed outcomes are achieved, would your organization be willing to evaluate conversion to an annual paid agreement?

Pilot scorecard

Agree on measures before the pilot begins.

Area	Baseline to capture	Pilot success measure
Staff workflow burden	Minutes per case, manual list maintenance, report-preparation time	Fewer manual steps or measured reduction in avoidable staff effort
Outreach reliability	On-time contact rate, missed outreach, visibility of due work	Improved visibility and completion within the agreed cadence
Family activation	Existing handoff method and response rate	Defined share of invited families reaches a first meaningful action
Family usefulness	Existing family questions and feedback	Families report clearer next steps and less administrative confusion
Implementation feasibility	Training constraints, systems, workflow complexity	Launch occurs within an agreed timeline without excessive staff burden

Do not promise a specific 50–70% labor reduction or any other numerical result before measuring it. In early materials, call results **pilot hypotheses**.

Legal, privacy, and safety checklist

Obtain professional advice on these issues

- Form the legal entity and document founder ownership, intellectual-property assignment, contractor agreements, and basic governance.
- Conduct trademark clearance for KinPath and fallback names before investing in brand assets.
- Review Georgia funeral-service and consumer-protection boundaries before coordinating vendors, arranging services, quoting prices, accepting referral fees, or charging a coordination fee.
- Establish the boundary between legal information and legal advice for probate, estate, benefit, and claim content.
- Map data flows for consumer-directed signup, partner-sponsored referral, and future integrated partner use.
- Determine when a BAA or more formal data agreement may be required based on actual data flows and contracts.
- Prepare a design-partner agreement, pilot agreement, terms of use, privacy notice, confidentiality terms, data-processing terms, and acceptable-use policy.

Build the minimum trust foundation

- Use role-based, least-privilege access.
- Require multi-factor authentication for staff and administrator accounts.
- Encrypt data at rest and in transit.
- Maintain audit logs for internal access and key user activity.
- Use secure backups and document a recovery approach.
- Collect only the minimum information necessary for the first workflow.
- Avoid collecting Social Security numbers, bank-account details, full financial records, or full health records unless a validated use case requires them.
- Create retention and deletion rules before launch.
- Design a basic incident-response plan and assign an owner for security/privacy decisions.
- Do not use sensitive data for advertising or to train public AI models.

Family and crisis safeguards

- Use “workspace administrator” rather than language implying that a user is automatically an executor, legal next of kin, or authorized decision-maker.
- Log invitations, permissions, access changes, and factual corrections.
- Preserve audit history but allow errors to be corrected transparently.
- For conflict-sensitive decisions, provide neutral information and encourage identification of the legally authorized decision-maker or consultation with qualified counsel.
- Do not attempt to arbitrate family disputes.
- Include prominent crisis-support and emergency routing. For U.S. users in immediate emotional crisis, provide access to 988 and local emergency options.
- Use plain language, readable design, larger text options, and a multilingual-ready structure.

Funding plan

Raise in stages

Stage	Funding target	What it funds	Evidence needed
Validation	$5,000–$25,000	Research, prototype, limited legal work, landing page, partner outreach	Interviews, recurring workflow problem, prototype feedback
Pilot preparation	$50,000–$150,000	Narrow secure web MVP, legal/privacy baseline, 2–3 hospice pilots, onboarding	3–5 LOIs, pilot scorecard, named customer champions
Institutional-ready pre-seed	$450,000–$650,000	12–15 months of engineering, security, implementation, customer success, and repeatable hospice sales	Live use, measured outcomes, paid conversion or renewal evidence

Sources to pursue

- Customer-funded pilots
- Carefully structured friends-and-family funding, only with full risk disclosure and proper documents
- HealthIT-focused accelerators and startup programs in Atlanta
- Non-dilutive grant programs where KinPath can articulate a true research/development question
- HealthIT angels and early-stage investors
- Strategic relationships with hospice, senior-care, benefits, and care-navigation organizations
- Pitch competitions for visibility, non-dilutive prize money, and warm investor introductions

Fundraising materials to complete by day 60

- 10-slide investor deck
- One-page company overview
- Product prototype link or short demo recording
- Customer-discovery summary
- Current-state and future-state workflow map
- Design-partner LOIs
- Pilot scorecard
- 12-month lean operating budget
- Use-of-funds plan for $50,000, $150,000, and $550,000 scenarios
- Legal/privacy/data-flow one-pager
- Founder biography and personal reason for building the company

Suggested investor pitch

> KinPath is a bereavement-operations and family-guidance platform for hospices. Hospice teams must manage structured bereavement follow-up, family questions, outreach documentation, and a difficult transition after a patient dies, often through fragmented manual workflows. KinPath gives staff an auditable workflow and gives families a calm, guided plan for what comes next. We are validating the product with Atlanta hospice design partners and measuring workflow burden, outreach reliability, family activation, and implementation feasibility.

Days 61–90: Prepare the smallest credible pilot

Select your technical approach

Start with a secure responsive web application.

The first production MVP should include:

- Hospice staff dashboard
- Basic case management and outreach schedule
- Contact logging
- Family invitation and activation
- First-72-hours family guidance
- Task delegation
- Secure document request and receipt tracking
- Permissions appropriate to the initial workflow
- Reporting/export capability
- Security, audit logging, and privacy basics

Do not start with native apps unless pilot users prove they are essential.

Select a technical lead or builder carefully

Before hiring anyone:

- Require references from prior clients or employers.
- Confirm healthcare-adjacent security and privacy competency.
- Use a fixed scope and milestone-based contract.
- Ensure all code, design files, infrastructure accounts, documentation, and intellectual property are owned by KinPath.
- Avoid giving large equity merely for app development unless the person is a genuinely committed, long-term cofounder with complementary leadership responsibilities.
- Require clear security practices and a documented handoff.
- Do not allow a vendor to place essential accounts, source code, or cloud infrastructure under its personal ownership.

Pilot implementation plan

For each pilot hospice:

1. Confirm executive sponsor and frontline champion.
2. Map the existing outreach cadence and staff responsibilities.
3. Agree on the pilot case population and activation procedure.

4. Establish data boundaries and permitted data flows.
5. Train the limited initial user group.
6. Capture baseline metrics before launch.
7. Launch with a small cohort rather than every case at once.
8. Conduct weekly check-ins during the first month.
9. Track bugs, workarounds, training gaps, adoption barriers, and family feedback.
10. Hold a formal pilot review with the conversion decision-maker.

Final execution checklist

1. Write the one-page KinPath product brief.
2. Build a 40-prospect Atlanta hospice list.
3. Create the discovery interview script and tracking system.
4. Send the first 10–15 outreach messages.
5. Complete at least 15 interviews by day 30.
6. Map five real or de-identified workflows.
7. Build a clickable prototype of one complete staff-to-family case journey.
8. Test the prototype with at least five hospice teams.
9. Secure three to five written design-partner commitments.
10. Establish baseline measures and a pilot scorecard.
11. Obtain focused legal, privacy, corporate, trademark, and Georgia marketplace-boundary guidance.
12. Build the investor deck, one-page overview, budget, and use-of-funds plan.
13. Apply to appropriate Atlanta HealthIT and startup programs.
14. Negotiate customer-funded pilots or raise a small pilot-preparation round.
15. Select a technical lead or builder using a narrow milestone-based scope.
16. Launch the smallest secure web MVP with two to three committed hospice pilots.

Decision rule

If you do not hear the same urgent workflow problem repeatedly, cannot obtain access to real workflow observation, or cannot secure serious design-partner commitments, do not spend heavily on development. Narrow the problem further or revise the customer wedge before raising more money.

The goal is not to prove that bereavement is difficult. Everyone already knows that. The goal is to prove that KinPath can make a specific hospice workflow more reliable for staff and more navigable for families—and that a buyer will pay for that outcome.